

Ultrafiltration, Hemoperfusion and Hemofiltration

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Tracking Information

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Manual Section Number

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Manual Section Title

Ultrafiltration, Hemoperfusion and Hemofiltration

Version Number

1

Effective Date of this Version

08/20/1987

Description Information

Benefit Category

Inpatient Hospital Services

Institutional Dialysis Services and Supplies

Outpatient Hospital Services Incident to a Physician's Service

Please Note: This may not be an exhaustive list of all applicable Medicare benefit categories for this item or service.

Item/Service Description

A. Ultrafiltration

This is a process for removing excess fluid from the blood through the dialysis membrane by means of pressure. It is not a substitute for dialysis. Ultrafiltration is utilized in cases where excess fluid cannot be removed easily during the regular course of hemodialysis. When it is performed, it is commonly done during the first hour or two of each hemodialysis on patients who, e.g., have refractory edema. Ultrafiltration is a covered procedure under the Medicare program (effective for services performed on and after September 1, 1979).

Predialysis Ultrafiltration

While the predialysis ultrafiltration procedure requires additional staff care, the facility dialysis rate is intended to cover the full range of complicated and uncomplicated nonacute dialysis treatments. Therefore, no additional facility charge is recognized for predialysis ultrafiltration. The physician's role in ultrafiltration varies with the stability of the patient's condition. In unstable patients, the physician may need to be present at the initiation of dialysis, and available either in- house or in close proximity to monitor the patient carefully. In patients who are relatively stable, but who seem to accumulate excessive weight gain, the procedure requires only a modest increase in physician involvement over routine outpatient hemodialysis.

Occasionally, medical complications may occur which require that ultrafiltration be performed separate from the dialysis treatment, and in these cases an additional charge can be recognized. However, the claim must be documented as to why the ultrafiltration could not have been performed at the same time as the dialysis.

Indications and Limitations of Coverage

B. Hemoperfusion

This is a process which removes substances from the blood using a charcoal or resin artificial kidney. When used in the treatment of life threatening drug overdose, hemoperfusion is a covered service for patients with or without renal failure. Hemoperfusion generally requires a physician to be present to initiate treatment and to be present in the hospital or an adjacent medical office during the entire procedure, as changes may be sudden. Special staff training and equipment are required.

Develop charges for hemoperfusion in the same manner as for any new or unusual service. One or two treatments are usually all that is necessary to remove the toxic compound; document additional treatments. Hemoperfusion may be performed concurrently with dialysis, and in those cases payment for the hemoperfusion reflects only the additional care rendered over and above the care given with dialysis.

The effects of using hemoperfusion to improve the results of chronic hemodialysis are not known. Therefore, hemoperfusion is not a covered service when used to improve the results of hemodialysis. In addition, it has not been demonstrated that the use of hemoperfusion in conjunction with deferoxamine (DFO), in treating symptomatic patients with iron overload, is efficacious. There is also a paucity of data regarding its efficacy in treating asymptomatic patients with iron overload. Therefore, hemoperfusion used in conjunction with DFO in treating patients with iron overload is not a covered service; i.e., it is not considered reasonable and necessary within the meaning of §1862(a)(1) of the Act.

However, the use of hemoperfusion in conjunction with DFO for the treatment of patients with aluminum toxicity has been demonstrated to be clinically efficacious and is therefore regarded as a covered service.

C. Hemofiltration

This is a process which removes fluid, electrolytes and other low molecular weight toxic substances from the blood by filtration through hollow artificial membranes and may be routinely performed in 3 weekly sessions. Hemofiltration (which is also known as

diafiltration) is a covered procedure under Medicare and is a safe and effective technique for the treatment of ESRD patients and an alternative to peritoneal dialysis and hemodialysis. In contrast to both hemodialysis and peritoneal dialysis treatments, which eliminate dissolved substances via diffusion across semipermeable membranes, hemofiltration mimics the filtration process of the normal kidney. The technique requires an arteriovenous access. Hemofiltration may be performed either in facility or at home.

The procedure is most advantageous when applied to high-risk unstable patients, such as older patients with cardiovascular diseases or diabetes, because there are fewer side effects such as hypotension, hypertension or volume overload.

Transmittal Information

Transmittal Number

18

Revision History

08/1987 - Revised to indicate that hemoperfusion is not covered when used in conjunction with deferoxamine (DFO) to remove iron overload and is covered when used in conjunction with DFO to treat aluminum toxicity. Also revised to cover hemofiltration for renal patients. Effective date 08/20/1987. (TN 18)

Additional Information

Other Versions

Title	Version	Effective Between	
Ultrafiltration, Hemoperfusion and Hemofiltration	1	08/20/1987 - N/A	You are here